BONUS 3 · PROFESSIONAL REFERENCE

Physical Assessment Guide

Standardised assessment tools, functional screening scales, cognitive screening overview, ADL measures, progress tracking templates, and SMART goal-setting guidance for senior activity professionals.

6 Sections | Tinetti / Berg / TUG | MMSE & Clock Drawing | Barthel & FIM | Tracking Templates | SMART Goals



SECTION

Introduction to Functional Assessment in Seniors

Understanding the role of assessment in senior activity programming

Why We Assess

Introduction to Functional Assessment in Seniors

Functional assessment forms the foundation of all effective senior activity and rehabilitation programming. Without a clear baseline understanding of an individual's current capabilities — physical, cognitive, and social — programmes risk being either too challenging, leading to injury or withdrawal, or insufficiently stimulating, failing to produce the meaningful progress that sustains engagement.

A well-conducted assessment respects the dignity of each participant, gathering information through structured observation, standardised tools, and collaborative conversation. It is not a test to be passed or failed, but a professional conversation between the activity coordinator or therapist and the older adult — one that centres on capability, preference, and aspiration.

This guide introduces the most widely used standardised assessment tools in the United Kingdom and internationally, explains how each is administered and scored, and provides ready-to-use tracking templates so that progress can be documented consistently over time.

All assessments described in this guide should be conducted only by trained staff and with the informed consent of the participant. In residential or community settings, individual assessment data must be stored securely, shared only with the care team, and reviewed at regular intervals — typically quarterly or after any significant health event such as a fall, hospitalisation, or change in medication.

Principles of Senior Assessment

Introduction to Functional Assessment in Seniors

The following core principles underpin every assessment interaction described in this guide:

Person-Centred Approach

Assessment findings are meaningless without context. Always combine standardised scores with narrative — what does this person want to do that they currently cannot? What are their fears? What are their strengths? Goals set in partnership with the individual are more likely to be sustained and to produce genuine quality-of-life improvement.

Safety First

No assessment should place a participant at risk of injury. Before any physical assessment, observe posture, skin colour, and breathing at rest. Ask about pain. Know the individual's medical history and contraindications. Have a chair immediately available for all standing assessments. Stop and reassess if the participant shows any sign of distress, dizziness, or pain.

Cultural Sensitivity

Language barriers, cultural attitudes toward testing, literacy levels, and sensory impairments all affect assessment performance. Use interpreters when needed. Never draw conclusions about cognitive status from culturally biased or linguistically inaccessible tools. Adapt administration methods for visual or hearing impairment.

Regular Reassessment

A single assessment is a snapshot. Progress — and decline — are only visible through consistent, repeated measurement over time. Build reassessment intervals into care and activity plans from the outset.

SECTION

Balance & Gait Assessment Scales

Tinetti, Berg Balance Scale, and Timed Up and Go

Tinetti Performance-Oriented Mobility Assessment (POMA)

Balance & Gait Assessment Scales

The Tinetti POMA, developed in 1986, remains one of the most widely used clinical tools for assessing fall risk in community-dwelling and institutional older adults. It comprises two subscales: a Balance Subscale (9 items, maximum 16 points) and a Gait Subscale (7 items, maximum 12 points), yielding a total score of up to 28 points.

Interpretation

Total Score	Fall Risk	Recommended Action
< 19	High risk of falls	Urgent referral; comprehensive fall prevention programme
19–24	Moderate risk	Balance and gait training; home hazard assessment
25–28	Low risk	Maintenance exercise; regular reassessment every 6 months

Administration Notes

The balance subscale is assessed in a standardised chair without armrests. Gait is observed over a 3-metre walk. The assessor rates each item based on observed performance. Items should not be prompted or coached during scoring, though the participant should be told what they are being asked to do in plain language. Allow adequate warm-up time before assessment. Record the type of walking aid used.

Berg Balance Scale (BBS)

Balance & Gait Assessment Scales

The Berg Balance Scale comprises 14 functional tasks scored 0–4 each (total maximum 56 points). It was designed to assess static and dynamic balance in older adults and has high interrater reliability when assessors are trained.

Selected Items

- **Sitting to standing** — ability to rise without hands
- **Tandem standing** — ability to hold heel-to-toe for 30 seconds
- **Single-leg stance** — ability to stand on one leg for up to 10 seconds
- **360-degree turn** — ability to turn a full circle safely
- **Retrieving object from floor** — picking up a shoe from the floor

Scoring Guide

Score Range	Balance Level	Clinical Implication
0–20	High fall risk	Wheelchair use may be indicated; intensive therapy
21–40	Medium fall risk	Supervised ambulation; targeted balance training
41–56	Low fall risk	Independent but monitor; preventive exercise recommended

Note: A change of 4+ points represents a meaningful clinical difference in community-dwelling elders.

Timed Up and Go (TUG)

Balance & Gait Assessment Scales

The Timed Up and Go test is perhaps the most practical quick-screen for fall risk in clinical and community settings. The participant stands from a standard chair, walks 3 metres, turns, walks back, and sits. Time is recorded in seconds.

Normative Reference Values

Age Group	Normal (s)	Concern (s)	High Risk (s)
60–69	< 9	9–12	> 12
70–79	< 11	11–14	> 14
80+	< 14	14–20	> 20

Dual-Task TUG

A validated enhancement is the Dual-Task TUG: the participant carries a full cup of water or counts backward while completing the standard TUG. A meaningful increase in time (≥ 4 seconds) compared to the standard TUG indicates that attentional demands significantly affect gait safety — an important finding for activity planning.

Record the type of walking aid used and any significant observations: stumbles, near-misses, hesitation on turning.

Functional Reach Test

Balance & Gait Assessment Scales

The Functional Reach Test measures the maximum distance an individual can reach forward with an outstretched arm while standing, expressed in centimetres or inches. It provides a quick, objective measure of the limits of stability and correlates well with fall frequency in older adults.

Procedure

Mount a measuring tape horizontally on the wall at shoulder height. The participant stands beside the wall (dominant side), arm raised at 90 degrees. Record the starting position. The participant reaches as far forward as possible without stepping. Record the ending position. Subtract to get reach distance. Average of 3 trials is recorded.

Reference Values

Score	Interpretation
> 25 cm	Low risk of falls
15–25 cm	Moderate risk — monitor
< 15 cm	High risk — referral recommended

SECTION

Cognitive Screening Overview

MMSE, Clock Drawing Test, and considerations for activity planning

Mini-Mental State Examination (MMSE)

Cognitive Screening Overview

The MMSE, developed by Folstein et al. (1975), is a 30-point screening instrument assessing orientation, registration, attention, recall, language, and visuospatial function. It takes approximately 10 minutes to administer.

Domain Breakdown

Domain	Max Points	Example Items
Orientation	10	Day, date, month, year, season; country, county, town, location, floor
Registration	3	Repeat 3 words (apple, table, penny)
Attention & Calculation	5	Serial 7s or spell WORLD backwards
Recall	3	Recall the 3 words from Registration
Language	9	Naming, repetition, 3-step command, reading, writing, copying

Score Interpretation

Score	Interpretation
24–30	Normal cognitive function
18–23	Mild cognitive impairment — plan accordingly
0–17	Severe cognitive impairment — specialist referral

Important caveat: MMSE scores are significantly affected by education level, cultural background, and sensory impairment. A low score does not automatically indicate dementia and must be interpreted in full clinical context. The MMSE is a screen, not a diagnosis.

Clock Drawing Test (CDT)

Cognitive Screening Overview

The Clock Drawing Test is a brief, widely used cognitive screening tool that assesses executive function, visuospatial ability, and constructional praxis. It is particularly sensitive to early dementia and frontal lobe dysfunction.

Administration

Provide the participant with a pre-drawn circle (or ask them to draw a clock face). Instruct them to: (1) draw numbers on the clock face, and (2) draw hands to show 10 minutes past 11 (or 3:40).

Scoring (10-point scale)

- Points 1–2: Correct spacing of hour numbers
- Points 3–4: All 12 numbers present
- Points 5–6: Hands present
- Points 7–8: Correct time depicted
- Points 9–10: Minute hand clearly distinguishable from hour hand

Scores below 7/10 warrant further assessment. Common error patterns provide useful diagnostic clues: crowding numbers to one side suggests visuospatial difficulties; placing hands incorrectly despite correct numbers may indicate executive dysfunction.

The CDT should be used alongside, not instead of, a full cognitive assessment battery.

Implications for Activity Planning

Cognitive Screening Overview

Cognitive screening results directly inform how activity programmes are structured and delivered. The following principles apply:

Mild Impairment (MMSE 18–23)

Participants with mild impairment benefit from structured activities with clear, simple instructions given one step at a time. Environmental cues help (large-print labels, consistent room layout). Activities should tap long-term memory, which is typically preserved longer than working memory. Reminiscence, music, and craft activities are particularly well suited.

Moderate Impairment (MMSE 12–17)

Focus on sensory stimulation, music, gentle movement, and 1:1 interaction. Keep sessions shorter (20–30 minutes). Use familiar objects and themes. Avoid timed tasks or activities that highlight memory failure.

Severe Impairment (MMSE below 12)

Meaningful engagement is still possible and important. Tactile stimulation, hand massage, music from the person's era, and gentle movement can reduce agitation and support wellbeing even when verbal communication is minimal. Staff training in dementia-specific communication is essential.

SECTION

Mobility & ADL Assessment

Barthel Index and Functional Independence Measure

Barthel Activities of Daily Living Index

Mobility & ADL Assessment

The Barthel Index, first described in 1965, assesses independence in 10 activities of daily living. It is the most widely used ADL measure in rehabilitation and long-term care settings worldwide. Each item is scored based on the level of assistance required, yielding a total of 0–100 (or 0–20 in the modified version).

Barthel Index Items

Activity	Max Score	Key Levels
Feeding	10	0=dependent; 5=needs help cutting; 10=independent
Bathing	5	0=dependent; 5=independent
Grooming	5	0=needs help; 5=independent
Dressing	10	0=dependent; 5=needs help; 10=independent
Bowels	10	0=incontinent; 5=occasional accident; 10=continent
Bladder	10	0=incontinent; 5=occasional accident; 10=continent
Toilet use	10	0=dependent; 5=needs some help; 10=independent
Transfers (chair/bed)	15	0=unable; 5=major help; 10=minor help; 15=independent
Mobility	15	0=immobile; 5=wheelchair; 10=walks with help; 15=independent
Stairs	10	0=unable; 5=needs help; 10=independent

A total score of 60 or above generally indicates the ability to live at home with some support. Scores below 40 suggest significant dependence.

Functional Independence Measure (FIM)

Mobility & ADL Assessment

The FIM is a more detailed 18-item assessment tool covering motor (13 items) and cognitive (5 items) dimensions of ADL performance. It uses a 7-level scale from 1 (total assistance) to 7 (complete independence) for each item, yielding a maximum of 126 points.

FIM Motor Items

Self-care (6): eating, grooming, bathing, upper dressing, lower dressing, toileting. Sphincter control (2): bladder, bowel. Transfers (3): bed/chair/wheelchair, toilet, tub/shower. Locomotion (2): walk/wheelchair, stairs.

FIM Cognitive Items

Communication (2): comprehension, expression. Social cognition (3): social interaction, problem-solving, memory.

Using FIM in Activity Planning

The FIM is primarily a rehabilitation discharge and outcome tool. In activity programming, the key value is identifying where functional gains are possible. Participants scoring 4–5 on transfers (requiring supervision or minimal contact assistance) are often prime candidates for targeted sit-to-stand and gait training programmes included in this vault.

Note: formal FIM administration requires certified training. Activity coordinators should work with rehabilitation teams who can provide FIM data to inform programme design.

Integrating Assessment into Programme Design

Mobility & ADL Assessment

The various assessment tools described in this guide are most valuable when their findings are actively used to shape individual and group activity programming. The following integration framework is recommended:

Step 1: Baseline Assessment

On admission or at programme start, administer the TUG, Barthel Index, and a brief cognitive screen (MMSE or CDT). Record results in the tracking templates (Section 5). Identify each participant's Level: 1 (Independent), 2 (Assisted), or 3 (Supported) using the framework described below.

Step 2: Activity Level Assignment

Assessment Profile	Activity Level	Suitable Programme
TUG < 12s; Barthel ≥ 80; MMSE ≥ 24	Level 1 – Independent	Standing balance, gait, cognitive, group activities
TUG 12–20s; Barthel 40–79; MMSE 18–23	Level 2 – Assisted	Seated balance, adapted mobility, simplified cognitive
TUG > 20s or unable; Barthel < 40; MMSE < 18	Level 3 – Supported	Bedside/wheelchair, sensory, 1:1 or small group

Step 3: Quarterly Reassessment

Repeat core assessments every 3 months or after any significant health event. Document changes in the tracking templates. Update activity level assignments and programme goals accordingly.

SECTION

Progress Tracking Templates

Printable forms for individual participant monitoring

Individual Assessment Record — Page 1

Progress Tracking Templates

Participant Name: _____ Date of Birth: _____

Assessor: _____ Assessment Date: _____

Medical Notes / Contraindications: _____

Assessment	Score	Date	Notes
TUG (seconds)			
TUG Dual-Task			
Functional Reach (cm)			
Berg Balance Score			
Tinetti Total			
MMSE Score			
Clock Drawing			
Barthel Index			

Activity Level Assigned (circle): LEVEL 1 — Independent LEVEL 2 — Assisted LEVEL 3 — Supported

Recommended Programme: _____

Goals (3-month): _____

Reviewed by: _____ Signature: _____

Individual Assessment Record — Page 2: Quarterly Progress

Progress Tracking Templates

Measure	Baseline	Q1	Q2	Q3	Q4
TUG (s)					
Functional Reach (cm)					
Berg Score					
MMSE					
Barthel Index					
Activity Level					
Falls in period					

Notable Changes / Clinical Events: _____

Programme Adjustments Made: _____

Clinician Sign-Off: _____

Group Session Attendance & Participation Log

Progress Tracking Templates

Date	Session Type	Participants	Level Mix (1/2/3)	Key Observations	Incidents

Month: _____ Programme: _____ Lead: _____

Monthly Fall Incident Log

Progress Tracking Templates

Date	Participant Initials	Time	Location	Activity at Time	Injuries	Action Taken

Month: _____ Facility: _____

Total falls this month: ___ Review meeting date: _____ Attendees: _____

Programme changes following review: _____

SECTION

Goal Setting Guide

Collaborative SMART goal setting for senior activity programmes

SMART Goal Framework for Activity Programmes

Goal Setting Guide

Effective goal setting transforms assessment findings into meaningful, motivating targets for both participant and practitioner. The SMART framework — Specific, Measurable, Achievable, Relevant, Time-bound — provides the structure for goals that are clinically meaningful and personally motivating.

The SMART Framework Applied

Element	Question to Ask	Example (Poor → SMART)
Specific	What exactly will the participant do?	Poor: "improve balance" → SMART: "stand on one leg for 10 seconds"
Measurable	How will progress be tracked?	Poor: "walk better" → SMART: "complete TUG in under 14 seconds"
Achievable	Is this realistic given current assessment?	Poor: "run 5km" → SMART: "walk 200 metres without stopping"
Relevant	Does it connect to the person's own priorities?	Poor: "improve grip strength" → SMART: "open a jar of jam independently"
Time-bound	By when?	Poor: "get stronger" → SMART: "by the 3-month review"

Collaborative Goal-Setting Process

1. Share assessment results in plain language — avoid jargon. 2. Ask: 'What would you most like to be able to do that you currently find difficult?' 3. Explore what achieving that goal would mean to the person's life. 4. Frame the clinical goal in terms of the personal goal. 5. Write the goal with the participant, not for them. 6. Review at the agreed interval and celebrate progress.

Goal Review Template & Discharge Planning

Goal Setting Guide

Participant: _____ Review Date: _____

Goal 1: _____ Target Date: _____

Progress (circle): Not started In progress Achieved Revised Discontinued

Notes: _____

Goal 2: _____ Target Date: _____

Progress (circle): Not started In progress Achieved Revised Discontinued

Notes: _____

Goal 3: _____ Target Date: _____

Progress (circle): Not started In progress Achieved Revised Discontinued

Notes: _____

Discharge / Transition Planning:

Reason for discharge: _____ Date: _____

Maintenance programme recommended: _____

Onward referral: _____

Home exercise programme provided: Yes / No Copy filed: Yes / No

Clinician: _____ Signature: _____